



2026 SALARY DATA

Nurse Practitioner Salary Guide

National median **\$129,210** per year (BLS OEWS, Nurse Practitioners (29-1171) — median annual wage, May 2024). Pay by experience level, practice setting, and specialty premium, plus what to check before you negotiate — for nurse practitioners. State-by-state figures update daily at nphiring.com/salary-guide.

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\$129,210

NATIONAL MEDIAN

BLS OEWS, Nurse Practitioners (29-1171)
— median annual wage, May 2024

45%

PROJECTED JOB GROWTH

BLS Employment Projections — Nurse
Practitioners (2022–2032)

27 states + DC

FULL PRACTICE AUTHORITY

AANP State Practice Environment

90 million+

**IN PRIMARY-CARE SHORTAGE
AREAS**

HRSA Bureau of Health Workforce,
Designated HPSA Quarterly Summary
(primary care)

Quick Answer: NP Salary in 2026

The median NP salary is **\$129,210 per year** (BLS OEWS, Nurse Practitioners (29-1171) — median annual wage, May 2024). New graduates start at \$95,000–\$115,000, while experienced NPs (7–15 years) earn \$150,000–\$180,000. Private practice owners can earn \$180,000–\$300,000+. Pay runs highest in West Coast and Northeast markets, with California, Washington, and New Jersey near the top in federal wage data.

Methodology & Sources

Every national figure in this guide traces to a named public source; every posting-derived figure traces to live listings on NP Hiring. Live inventory counts come from 806 active NP postings with a disclosed salary on NP Hiring (snapshot 2026-07-29); posting data updates daily. This guide reports those counts and the board's published salary-validation bands rather than a frozen state-by-state wage table — see “Salary by Location”.

- BLS OEWS, Nurse Practitioners (29-1171) — median annual wage, May 2024 — <https://www.bls.gov/oes/current/oes291171.htm> (as of 2024-05)
- BLS Employment Projections — Nurse Practitioners (2022–2032) — <https://www.bls.gov/ooh/healthcare/nurse-anesthetists-nurse-midwives-and-nurse-practitioners.htm> (as of 2024)
- AANP State Practice Environment — <https://www.aanp.org/advocacy/state/state-practice-environment> (as of 2025)
- HRSA Bureau of Health Workforce, Designated HPSA Quarterly Summary (primary care) — <https://data.hrsa.gov/topics/health-workforce/shortage-areas> (as of 2025)

Experience, setting, and specialty ranges reflect the published market bands on nphiring.com/salary-guide. Individual offers vary by employer, location, credentials, and experience — treat every range here as a starting point for negotiation, not a guarantee.

Where NP Pay Comes From — and Where to Look It Up

As of 2026-07-29, NP Hiring lists **806** active NP postings with a disclosed salary, from **79** hiring employers across **50** states, out of 1,274 published openings in total.

Why this guide does not print a state-by-state wage table. State pay on NP Hiring is derived from live postings that change every day, and the disclosed-salary pool mixes W-2 salaries with annualized contract and locum hourly rates — so a snapshot frozen into a PDF would go stale and would overstate typical pay. The **state-by-state table updates daily and is free at nphiring.com/salary-guide**, with a dedicated page per state. For an authoritative national wage benchmark, use the federal figure cited throughout this guide: **\$129,210** (BLS OEWS, Nurse Practitioners (29-1171) — median annual wage, May 2024).

NP Hiring Salary-Validation Bands

These are the board’s **ingestion thresholds**, not a measurement of NP pay. Every salary an employer posts is checked against them before the listing goes live: numbers outside the outer bounds are rejected as data-entry errors, and numbers outside the middle band are flagged for a human to look at. They are set deliberately wide so that legitimate edge cases are not thrown away, which means they describe what this board will *accept* — not what a typical NP earns. For a measured wage figure, use the federal benchmark above (**\$129,210**, BLS OEWS, Nurse Practitioners (29-1171) — median annual wage, May 2024); for market ranges, use the experience, setting and specialty tables in the next section.

BAND	RANGE	HOW THE BOARD USES IT
W-2 annual salary	\$60,000 – \$400,000	Accept/reject bounds for a posted W-2 figure. The ceiling is set high enough to admit the best-paid advanced-practice listings (CRNA-level roles in high-cost markets), so it is an upper limit on accepted data — not the top of the NP market
Outlier-review band	\$110,000 – \$170,000	A posted salary outside this band is flagged for manual review before publishing. It is a screening threshold carried over from the board’s configuration — not a survey, and not a claim about where most offers land
Contract / 1099 hourly	\$40 – \$350 per hour	Validated as an hourly rate — do not annualize it to compare against a W-2 offer

A \$350/hour contract rate multiplies out to \$728,000 on paper, but carries no benefits, no paid time off, no employer payroll-tax contribution, and no guaranteed hours. Compare contract rates to contract rates.

SALARY BREAKDOWN

What Impacts Your Earnings

By Experience Level

EXPERIENCE	SALARY RANGE	TYPICAL ROLES
New Grad (0-1 yr)	\$95,000 - \$115,000	Staff NP, Outpatient Clinic
Early Career (1-3 yrs)	\$110,000 - \$130,000	Staff NP, Telehealth Provider
Mid-Career (3-7 yrs)	\$125,000 - \$150,000	Senior NP, Team Lead
Experienced (7-15 yrs)	\$150,000 - \$180,000	Clinical Director, Supervisor
Expert (15+ yrs)	\$180,000 - \$250,000+	Director, Consultant, Private Practice

By Practice Setting

SETTING	SALARY RANGE	NOTES
Private Practice (Owner)	\$180,000 - \$300,000+	Highest earning potential, requires business skills
Travel / Locum Tenens	\$150,000 - \$250,000	Includes housing, travel, higher hourly rates
Telehealth / Remote	\$130,000 - \$180,000	Growing rapidly, flexible schedules
Outpatient Clinic	\$120,000 - \$160,000	Most common setting, steady patient load
Hospital / Inpatient	\$115,000 - \$150,000	Often includes shift differentials, benefits
Community Health (FQHC)	\$100,000 - \$130,000	May qualify for loan forgiveness programs

Specialty Premiums

SPECIALTY	PREMIUM VS. BASELINE	NOTES
Acute Care / Hospitalist	+10-20%	AGACNP certification, hospital demand
Psychiatric-Mental Health	+10-20%	PMHNP-BC certification, provider shortage
Emergency / Urgent Care	+10-20%	Dynamic environment, flexible scheduling
Dermatology / Aesthetics	+10-25%	Procedure-driven, cash-pay revenue
Gerontology / Palliative	+5-10%	Growing aging population
Private Practice (Owner)	+20-40%	Higher risk, no benefits
Rural / Underserved	+10-15%	Often includes loan repayment

Full Practice Authority

27 states + DC grant FPA (AANP State Practice Environment). NPs in FPA states earn **12-15% more** on average.

✓ Full Practice Authority

- +12-15% salary premium
- Can own practice independently
- Full clinical independence

Restricted / Reduced

- Baseline salary
- Requires physician collaboration
- Physician oversight required

Factors Affecting NP Salary — and How to Negotiate

Six levers that move NP compensation. Evergreen guidance — the same factor breakdown published on nphiring.com/salary-guide.



Geographic Location

States with higher cost of living and greater demand (CA, NY, MA) typically offer 20-40% higher salaries than rural areas.



Experience Level

Entry-level NPs start around \$95-115k. With 5+ years experience, salaries can reach \$150-180k or more.



Practice Setting

Private practice and telehealth positions often pay more than hospital or community health settings.



Employment Type

1099 contractors and travel NPs often earn 20-50% more than W2 employees, though without traditional benefits.



Specialization

Subspecialties like acute care, emergency, psychiatric-mental health, or dermatology can command premium pay (+10-25%).



Negotiation

NPs who negotiate can often secure 5-15% higher starting salaries plus signing bonuses (\$5,000-\$30,000).

Cite This Guide

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